

Patient Encounter Documentation: SOAP Note Case Study

Written assignment. You document a simulated patient encounter the way you will be expected to document one in practice.

COURSE	WEIGHT	LENGTH	DUE
Clinical Procedures I	20% of final grade	900–1,200 words	

Why this assignment

Charting is the part of your job that follows the patient for years and the part a court reads first. Employers report that new medical assistants are clinically capable but document incompletely, use unapproved abbreviations, or record conclusions they are not licensed to make. This assignment is where you practice that, on paper, before it counts.

Learning outcomes assessed

- 1 Obtain and record a chief complaint and patient history using the patient's own words.
- 2 Document vital signs, anthropometric measurements and point-of-care test results accurately and in the correct units.
- 3 Organize a patient encounter into a complete, chronological SOAP note.
- 4 Apply approved medical terminology and abbreviations, and avoid those on the do-not-use list.
- 5 Work within the medical assistant scope of practice, and recognize where it ends.
- 6 Document patient education and follow-up instructions in a form the patient can act on.

Case scenario

Simulated patient — for coursework only. Any resemblance to a real patient is coincidental.

A 52-year-old male arrives at the family practice clinic for a scheduled three-month follow-up for type 2 diabetes mellitus and hypertension. You room the patient at 10:15.

He tells you: *"I've been getting these headaches in the morning, maybe three or four days a week for the last month. And my feet feel kind of numb, like pins and needles, mostly at night."* He says he ran out of his blood pressure medication about two weeks ago and has not refilled it. He is checking his blood glucose "sometimes, maybe twice a week." He works night shifts at a distribution center and reports sleeping about five hours a day.

Measurements you obtain:

- Height 5 ft 10 in · Weight 232 lb
- Temperature 98.4 °F oral · Pulse 88 regular · Respirations 18 · SpO₂ 97% on room air

- Blood pressure 158/96 left arm, seated, adult cuff. Repeat after five minutes: 154/94
- Random capillary blood glucose (point-of-care) 218 mg/dL
- Pain: reports headache 4/10 at time of visit

Current medications per the patient: metformin 1000 mg twice daily, lisinopril 20 mg daily (not taken for approximately two weeks), atorvastatin 20 mg at bedtime. Allergies: penicillin (rash). The provider will see him after you complete rooming.

What to submit

#	Component	Requirements	Length
1	SOAP note	A complete note with Subjective, Objective, Assessment and Plan clearly separated. Written as a chart entry, not as an essay. Include date, time and your signature line with credential.	~400 w
2	Documentation rationale	Explain three specific choices you made: one thing you quoted rather than paraphrased, one measurement you repeated or qualified, and one thing you deliberately did <i>not</i> write. Say why for each.	~300 w
3	Patient education note	The follow-up and education you would document and reinforce, at a reading level the patient can use. Include what you would escalate to the provider before the patient leaves.	~200 w
4	Scope & privacy reflection	Identify one statement a medical assistant may <i>not</i> record as an assessment in this case and rewrite it correctly. Name one HIPAA consideration specific to this encounter.	~150 w

Requirements and ground rules

- ☐ Use only abbreviations from the approved course list. Do not use anything on the do-not-use list (for example *u*, *QD*, *MSO4*, trailing zeros).
- ☐ Record objective data in the units given. Do not convert unless you state the conversion.
- ☐ Attribute subjective information to the patient. Use quotation marks where you quote.
- ☐ Do not diagnose, interpret results, or adjust medication. Documenting a finding is in scope; concluding what it means is not.
- ☐ No fabricated data. If something was not measured or not asked, do not invent it. Write what is missing.
- ☐ Submit as a single PDF or Word file. Typed, 11 pt, single spaced, your name and student ID in the header.
- ☐ Cite any source you use (textbook chapter, CDC, clinic policy) with author and year.

Use of AI tools

You may use AI tools to check grammar or to test your understanding. You may not have an AI tool write the SOAP note or the reflection for you. If you used an AI tool, say so at the end of your submission in one line: which tool, and what you used it for. Undeclared use is handled under the academic integrity policy.

How you will be graded

Part B of this document is the assessment form your instructor completes. Read it before you write. Seven criteria, each scored 1–4, weighted to 100 points. A passing submission scores **70 points or higher** and scores at least **Developing (2)** on Criterion 5, Scope of practice. A score of 1 on Criterion 5 is an automatic resubmission regardless of total, because a scope error is a patient safety error.

One resubmission is allowed within five working days of feedback. A resubmitted assignment is capped at 80 points.

SOAP Note Case Study: Grading and Feedback Form

Completed by the instructor. Returned to the student with the graded submission.

STUDENT NAME _____	STUDENT ID _____	INSTRUCTOR _____	DATE ASSESSED _____
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Rating scale

4 — Exemplary

Ready for the clinical setting with no correction.

3 — Proficient

Meets the standard. Minor issues, no patient risk.

2 — Developing

Partially meets it. Would need correction before filing.

1 — Beginning

Does not meet it. Would be unsafe or unusable as a record.

Criteria

Criterion	4 — Exemplary	3 — Proficient	2 — Developing	1 — Beginning	Wt	Score
1. Subjective data Chief complaint, history, patient's own words	Chief complaint, onset, duration, frequency and context all captured. Patient's words quoted where it matters. Medication gap and sleep pattern both recorded.	Chief complaint and main history present and attributed to the patient. One detail thin or paraphrased where a quote was better.	Chief complaint present but history incomplete. Patient statements rewritten as the writer's own conclusions.	No usable chief complaint, or subjective and objective data mixed together.	15	
2. Objective data Vitals, measurements, point-of-care results	All values recorded accurately with units, site, position and cuff size. Repeat blood pressure documented with the interval. Allergy status recorded.	All values present and accurate. One qualifier missing (site, position or timing).	Values present but with errors, missing units, or the repeat reading omitted.	Values wrong, invented, or largely absent.	20	

Criterion	4 — Exemplary	3 — Proficient	2 — Developing	1 — Beginning	Wt	Score
3. Note structure SOAP organization and chronology	Four sections clearly separated, each holding only what belongs in it. Chronological, dated, timed and signed with credential.	Four sections present and mostly correct. One item filed in the wrong section.	Sections blurred or out of order. Date, time or signature missing.	No recognizable SOAP structure. Reads as an essay.	15	
4. Plan & patient education Follow-up, instructions, escalation	Education is specific, actionable and at the patient's level. Correctly identifies what must be escalated to the provider before the patient leaves.	Education documented and appropriate. Escalation identified but not clearly prioritized.	Education generic ("advised to eat healthy"). Escalation vague or missing.	No education documented, or advice given that is outside scope.	15	
5. Scope of practice Safety-critical. See note below.	Stays fully within the MA role. Correctly identifies and rewrites the out-of-scope statement. Names a relevant HIPAA consideration for this encounter.	Stays within scope. Identifies the out-of-scope statement; rewrite is slightly off or HIPAA point is generic.	One borderline interpretive statement in the note, or the reflection does not identify the correct issue.	Diagnoses, interprets results, or advises a medication change. Automatic resubmission.	15	
6. Terminology & abbreviations	Correct terminology throughout. Approved abbreviations only. Nothing from the do-not-use list.	Correct throughout with one minor terminology slip that does not change meaning.	Several terminology errors, or an unapproved abbreviation used.	Terminology errors that change clinical meaning, or do-not-use abbreviations present.	10	
7. Professional writing Clarity, grammar, format, sources	Concise and unambiguous. Clean grammar and spelling. Format and length met. Sources cited.	Clear and readable. A few surface errors. Format met.	Errors slow the reader or create ambiguity. Format or length not met.	Hard to follow. Errors would make the record unreliable.	10	

Score

Weighted total (of 100)	Result	Resubmission required
	☐ Pass (≥ 70) ☐ Fail (< 70)	☐ No ☐ Yes — due by _____

Weighted total = (criterion score $\div$ 4) $\times$ weight, summed. A score of 1 on Criterion 5 requires resubmission regardless of the total.

Feedback

What worked

What to fix, in order of priority

What to do differently on the next encounter

Instructor signature

Date

Student acknowledgement